TEST REPORT

MC-7040

NAME	**: Miss. AKSHARA S R**	**LAB NO.**	: **MLK-7555**
AGE/SEX	: 21 Years / Female	COLLECTED ON	: 14-Sep-2026 12:19 pm
PHONE NO	:	RECEIVED ON	: 14-Sep-2026 12:33 pm
REFD Dr	: Dr.BLESSY V MATHEW	REPORTED ON	: 14-Sep-2026 02:57 pm **Final**
ADDRESS	:		
COLLECTED AT	JOHN MARIANS HOSPITAL		

Test Description	Results	Units	Biological Reference Interval
TSH;ThyroidStimulating Hormone Method : Electrochemiluminescence Sample : Serum	0.87	µIU/mL	0.27 - 4.2

Hypersensitive hTSH Assay.
TSH levels are subject to circadian variation,reaching peak levels between 2 am - 4.a.m and at a minimum between 6 pm -10 pm.The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations. Recommended test for T3 and T4 is unbound fraction or free levels as it is metabolically active.Physiological rise in Total T3 / T4 levels is seen in pregnancy and in patients on steroid therapy.
Interpretation :
TSH results between 4.2 to 15 show considerable physiologic & seasonal variation, suggest clinical correlation or repeat testing with fresh sample .
TSH results between 0.1 to 0.45 require correlation with patient age & clinical symptoms. As with increasing age, there are marked changes in thyroid hormone production, metabolism & its actions resulting in an increased prevalence of subclinical thyroid disease .TSH values may be transiently altered because of non thyroidal illness like severe infections,liver disease, renal and heart failure,severe burns, trauma and surgery etc .Drugs that decrease TSH values e.g:L-dopa,Glucocorticoid Drugs that increase TSH values e.g Iodine,Lithium,Amiodarone.
Cord Blood : 2.3 - 13.2 ulU/ml
Pregnancy:
1st Trimester : 0.1 - 2.5 ulU/ml
2nd Trimester : 0.2 -3.0 ulU/ml
3rd Trimester : 0.3 - 3.0 ulU/ml

** End Of Report **

Verified by:

Sreelatha P
Sr. Lab Technologist

Authorized by:

Dr.Minnu Elza Varghese
MBBS, MD (pathology)
Consultant Pathologist

Processed at : @Kozhencherry, M-Square Building, Opp. District Govt. Hospital 9526207700

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Test Description	Results	Units	Biological Reference Interval
Ferritin Method : CLIA Sample : Serum	6.3	ng/ml	11.0 - 307

Comment: Rechecked And Confirmed

Note: Increase in serum ferritin due to inflammatory conditions (Acute phase response) can mask a diagnostically low result
Comments
Serum ferritin appears to be in equilibrium with tissue ferritin and is a good indicator of storage iron in normal subjects and in most disorders. In patients with some hepatocellular diseases, malignancies and inflammatory diseases, serum ferritin is a disproportionately high estimate of storage iron because serum ferritin is an acute phase reactant. In such disorders iron deficiency anemia may exist with a normal serum ferritin concentration. In the presence of inflammation, persons with low serum ferritin are likely to respond to iron therapy.
Increased Levels

· Iron overload - Hemochromatosis, Thalassemia & Sideroblastic anemia
· Malignant conditions - Acute myeloblastic & Lymphoblastic leukemia, Hodgkin's disease & Breast carcinoma
· Inflammatory diseases - Pulmonary infections, Osteomyelitis, Chronic UTI, Rheumatoid arthritis, SLE, burns
· Acute & Chronic hepatocellular disease

Decreased Levels
Iron deficiency anemia

Anti Thyroid Antibodies (X-TPO +X-TG)

Anti TPO / Anti Microsomal Antibody Method : CLIA Sample : Serum	0.8	IU/ml	< 9.0 IU/ml
Anti Thyroglobulin Antibody; Anti Tg Method : CLIA Sample : Serum	0.01	IU/mL	< 4.11 IU/mL
IgE; Immunoglobulin ;Serum Method : Electrochemiluminescence Sample : Serum	99.8	IU/mL	0 - 100

Normal levels of IgE do not rule out possibility of IgE dependent allergies as the diagnostic sensitivity of the test depends upon elapsed time between exposure to an allergen and testing, patient age and affected target organs. No close correlation has been demonstrated between severity of allergic reaction and IgE levels.
Comments
Immunoglobulin E (IgE) is the most important trigger molecule for allergic information. The level of IgE is low during the first year of life, gradually increases with age and reaches adult levels after 10 years. As IgE is a mediator of allergic response, quantitative measurement can provide useful information for differential diagnosis of atopic and non-atopic disease. Patients with atopic diseases like Allergic asthma, Allergic rhinitis & Atopic dermatitis have moderately elevated IgE levels.
Increased Levels - Atopic/Non-atopic allergy, Hyper IgE syndrome, Parasitic infections, IgE Myeloma, Pulmonary Aspergillosis, Immunodeficiency states & Autoimmune diseases

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Test Description	Results	Units	Biological Reference Interval
Vitamin D,25-Hydroxy(VTD,TOTAL) Method : Electrochemiluminescence Sample : Serum	21.30	ng/ml	Severe Deficiency :< 10ng/ml Insufficiency : 10-30 ng/ml No Deficiency : > 30 ng/ml Toxicity : > 70 ng/ml

25(OH) vitamin D will be a mixture of the D2 and D3 forms, both the vitamin D2 and vitamin D3 forms of vitamin D must be measured to accurately assess total 25(OH) vitamin D levels.
Vitamin D promotes absorption of calcium and phosphorus and mineralization of bones and teeth. Deficiency in children causes Rickets and in adults leads to Osteomalacia. It can also lead to Hypocalcemia and Tetany. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs).
Decreased Levels
Inadequate exposure to sunlight , Dietary deficiency ,Vitamin D malabsorption ,Severe Hepatocellular disease , Drugs like Anticonvulsants,Nephrotic syndrome.
Increased levels
Vitamin D intoxication

** End Of Report **

Verified by:

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Sr. Lab Technologist

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Kindly correlate clinically

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